June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244-1850

Re: Health Care Transformation Task Force Comments on the Health Technology Ecosystem Request for Information *(CMS-0042-NC)*

The Health Care Transformation Task Force (HCTTF) is a non-profit collaborative that accelerates the pace of delivery system transformation to better pay for valuable care for patients. Representing a diverse set of organizations from across the industry – including providers, payers, purchasers, and consumer/patient advocacy organizations – we share a common commitment to transform our respective businesses and clinical models to deliver better health through high-quality care at reduced costs. We strive to provide a critical mass of policy, operational, and technical support that, when combined with the work being done by the Centers for Medicare & Medicaid Services (CMS) and other public and private stakeholders, can increase the momentum of delivery system transformation.

HCTTF welcomes the opportunity to share our feedback on the Health Technology Ecosystem Request for Information (CMS-0042-NC). We offer these recommendations in the spirit of collaboration, with the goal of helping CMS use technology to improve quality and access while reducing costs. As an organization committed to advancing person-centered, value-based care (VBC), we recognize the critical role that technology plays in transforming health care delivery. We believe that thoughtful integration of emerging technologies can accelerate the shift to a more responsive, efficient, and sustainable health care system.

PATIENTS & CAREGIVERS

CMS seeks input on questions related to use cases and workflows that involve patients and caregivers. The Task Force appreciates the opportunity to provide input on the design and deployment of consumer-facing technologies that directly impact these users. Our comments focus on the need for a patient-centered approach to digital health, privacy concerns, and additional considerations for ensuring patients can access necessary digital tools.

The Task Force supports the Administration's efforts to increase patient and caregiver engagement with digital health technologies.[1] Adoption of digital health tech has been slow due to limited insurance reimbursement structures, lack of integration into health care systems, and low patient awareness. **Specifically, CMS and its partners can drive adoption by (1) promoting transparency around the cost, pricing, and utilization of health care services, (2) supporting efforts to strengthen and standardize interoperability across systems, (3) investing in patient education initiatives that help individuals make informed decisions about their care, (4) prioritizing patients' ease of use and alignment with patient and caregiver digital literacy levels, and (5) enabling patients to own their health care data.**

I. Patient Access & Education

The Task Force believes that digital health tools must be designed and implemented with a patient-centered approach, starting with an assessment of patients' digital literacy, language needs, and motivations for using such products.[2] Many existing mobile health applications are above the recommended reading level for patient materials, and many are not available in different languages. **CMS should engage patients in conversations about their care goals and level of comfort with technology before introducing digital solutions.** Products must also be consistently accessible and affordable for patients to improve outcomes long-term. One example of this is making (or continuing to make) the purchase of approved digital health products an eligible expense for flexible spending accounts (FSAs). **We recommend that CMS support the use of digital health navigators who can assess digital literacy and guide patients and caregivers in using these tools effectively.**

Improving accessibility to data also means addressing significant logistical barriers. Many Medicare beneficiaries, especially those in rural areas, lack access to broadband internet and modern devices such as smartphones or tablets. This digital divide has led to gaps in telehealth and digital health adoption. **To address these challenges, we urge CMS to work with Congress to ensure permanent coverage and reimbursement for audio-only telehealth services, which remain a vital lifeline for many underserved communities – particularly rural patients. We also recommend federal support for programs that provide end-user devices and digital training to patients and caregivers, empowering them to confidently participate in their care using digital health technologies.**

II. Data Security & Privacy

Additionally, we emphasize the importance of data ownership and privacy. **Patients should have the ability to control and access their health data securely and easily.** Ensuring robust privacy

---

[1] PC–5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

[2] PC–6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

protections across the health care ecosystem will bolster public confidence in digital tools. A confidential physician-patient relationship remains critical to effective care, and federal efforts should aim to standardize privacy safeguards to ensure consistent protection of medical information across platforms and jurisdictions.

While data privacy is of high importance to patients, many third-party applications, wearable devices, and other consumer technologies that handle health data are not governed by the Health Information Portability and Accountability Act (HIPAA) or the Health Information Technology for Economic and Clinical Health (HITECH).[3] As a result, patients' sensitive health information accessed through these tools lacks the robust privacy and security protections afforded under these regulations.

In contrast, applications and portals developed and managed by HIPAA-covered entities – such as health care providers and payers – must comply with all relevant privacy, security, and health care regulatory requirements to ensure the security of patient data. **The Task Force recommends that CMS work toward extending similar consumer protections to data accessed or used by non-HIPAA-regulated entities, such as third-party technology companies. CMS and ASTP/ONC should develop a health app certification program to identify digital health tools and products that meet national standards for security, privacy, interoperability, and accessibility. The program should also explain benefits of use of the tools or product such as reduced out-of-pocket costs, faster appointment scheduling or proactive reminders for care.** Ensuring consistency in data protection standards across all digital health tools is essential to maintaining patient trust and safety.

PROVIDERS

CMS seeks input on questions related to use cases and workflows that involve providers. In our comments, we offer recommendations on how CMS and its partners can encourage provider adoption of digital health products, reduce administrative burdens, support interoperability, and protect the confidentiality and usability of health information.

III. Provider Education & Engagement

To increase provider adoption of digital health tools, **CMS should offer providers clear guidance and technical support that addresses core barriers, particularly concerns about liability.**[4] Uncertainty around compliance, documentation, and risk exposure limits providers' willingness to integrate digital products into their practice. These supports should be tailored to the specific needs of independent and smaller practices, which may lack the infrastructure to evaluate or implement digital health products without additional assistance.

---

[3] PC-4: What features are missing from apps you use or that you are aware of today?
[4] PR-1: What can CMS and its partners do to encourage providers to leverage approved digital health products for their patients?

Major barriers to adoption include insufficient broadband connectivity affecting 25% of rural communities, creating fundamental access limitations for internet-dependent digital health interventions. Patient-level challenges include limited digital health literacy, particularly among Medicare beneficiaries, and concerns about technology complexity. Provider knowledge gaps about FDA-approved digital products and uncertainty about appropriate clinical use cases hinder adoption. Additionally, unclear documentation requirements and uncertainty about how digital health tools integrate with existing quality measures create implementation hesitancy among providers.

CMS should focus on comprehensive provider education initiatives that distinguish FDA-approved digital therapeutics from wellness applications, emphasizing their evidence base and clinical integration potential. Key efforts should include developing educational resources with real-world clinical examples, creating implementation toolkits for different practice settings, and providing clear guidance on how digital health products fit within existing Medicare coverage and documentation requirements. **The Task Force also encourages CMS and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology's (ASTP/ONC) to consider offering financial assistance to small, independently owned physician practices that want to expand their health IT processes to assess and implement digital health technologies.**

CMS' guidance should also help providers communicate with patients about digital technologies. Providers should speak with patients about how their health information will be protected on these platforms. Providers should clearly explain the regulatory status of recommended digital health products, emphasizing whether they are FDA-cleared medical devices or general wellness tools. Essential information includes the specific clinical purpose, expected outcomes, and how the technology supports their individual treatment goals. Patients need comprehensive privacy information about data collection, storage, and sharing practices, along with their rights regarding health information.

Providers must assess patient-specific factors including clinical appropriateness, digital literacy capabilities, and technology access before recommending digital health tools. Ongoing clinical oversight is essential, requiring established protocols for reviewing generated data, responding to alerts, and maintaining care continuity. Documentation responsibilities include integrating digital health information into clinical records and treatment planning–this is often overlooked or deprioritized by providers due to the perceived and real difficulties of integrating with systems of record.

Digital products that handle protected health information should include guidance documentation that details what specific information is going to be collected, stored (if applicable), and transmitted or exchanged by the product, including information on any patient-facing options for sharing and exchange made available via the product. Such information transparency by digital product vendors is critically necessary to enable physicians' ability to share any necessary details with the patient about the digital health product being used. Providers should also underscore the process of scientific testing and assessment through clinical

trials, similar to medication or medical devices. These conversations can serve to strengthen the patient-provider relationship.

IV. Technological Integration in Clinical Workflows

A primary obstacle to the development of useful and effective applications is often the absence or belated inclusion of physicians, including primary care physicians, in the design, development, and real-world testing of applications.[5] There's a critical need for innovative applications that are intuitive, easily incorporated into existing clinical workflows, supportive of cognitive processing and medical decision making, and transparent regarding any algorithms used and health information being accessed.

The primary obstacle to innovative physician workflow applications lies in the complexity of integrating with existing EHR systems and the resulting workflow fragmentation. A multi-center study published in the Journal of the American Medical Informatics Association found eight-fold differences in clicks required for identical tasks across healthcare systems, with error rates for basic functions like medication ordering ranging from zero to thirty percent. This variability is part of a broader usability crisis: a national study of 5,197 physicians found EHRs scored 45.9 out of 100 on standardized usability measures – an "F" grade compared to Google Search (93) or Microsoft Excel (57).

Poor EHR design creates cascading barriers to innovation: developers must navigate fragmented, vendor-specific integration points while accommodating workflows that vary dramatically even within the same healthcare system. Each 1-point improvement in EHR usability correlates with 3% lower physician burnout odds, demonstrating how technical design failures directly impact both innovation adoption and clinician wellbeing.

Current EHR systems often lack standardized APIs for third-party applications, forcing developers to create custom integrations for each vendor platform. This fragmentation prevents scalable deployment of innovative solutions and increases development costs significantly. Additionally, many healthcare organizations struggle with inconsistent data formats and limited interoperability between systems, making it difficult for innovative applications to access the information needed to streamline workflows.

Mitigation strategies include accelerating adoption of standardized FHIR-based APIs with comprehensive implementation guidance, encouraging EHR vendors to develop more accessible developer programs with reduced integration barriers, and supporting healthcare organizations in adopting workflow optimization tools that can identify and address inefficient EHR usage patterns within their systems.

---

[5] PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

V. Quality Measurement

One of the most significant sources of administrative burden for family physicians is the variation in quality measurement and reporting requirements across payers.[6] Practices frequently engage with multiple insurers, each of which may have different expectations for what data are submitted, when, and how. Modest improvements by individual payers are insufficient if not standardized across the system. Additionally, providers in risk-bearing relationships also need greater transparency on cost variation, such as insights into how different provider groups are performing for different patient populations. This data on cost and quality would enable stakeholders to integrate best practices into their care models, supporting the administration's goals of reducing unnecessary spending and improving overall system efficiency. **To reduce this burden, CMS should promote alignment and standardization of quality measure definitions, submission formats, and timelines across both public and private payers.**

The use of Bulk Fast Healthcare Interoperability Resources (FHIR) data exports would reduce technical burden in transmitting quality data.[7] However, such solutions must be accompanied by aligned data definitions and measures across insurers – otherwise, the administrative complexity will remain. Similarly, leveraging registries for digital quality measurement can be beneficial if insurers agree to accept the registry's selected quality measures, which are often better aligned with specialty practice and quality improvement goals. **CMS should promote payer use of Bulk FHIR APIs to extract clinical quality data, shifting the calculation burden away from providers. CMS and ONC should work together to ensure certified health IT can support dual-purpose data reporting for use across MIPS, MA Stars, ACO programs, and payer-specific analytics.** Consolidating interoperability and quality reporting responsibilities – especially through aligned value-based payment models – will also help ensure that investments made by providers serve multiple purposes.

VI. Interoperability

Improving interoperability and reducing provider burden is necessary for providers to make high-quality, timely, and comprehensive decisions.[8] **CMS should consider payment models that reward interoperability efforts and the use of standardized data exchange formats, such as FHIR. Continued investment in the United States Core Data for Interoperability (USCDI) process and related acceleration efforts can further strengthen standardization. CMS should**

---

[6] PR–8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

[7] PR-8. What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?

[8] PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

also provide technical assistance—particularly for small and mid-sized practices—modeled after successful HITECH-era support programs. Importantly, interoperability must be embedded into clinical and administrative workflows so that external data becomes directly usable in day-to-day practice. **Finally, CMS should provide financial and operational support for regional health information exchanges (HIEs) and health data utilities (HDUs), which are critical enablers of data sharing at the local and regional levels.**

VII. Information Blocking

The Task Force supports ASTP/ONC's efforts to advance interoperability through information blocking regulations.[9] Access to actionable, timely health data is foundational to high-quality care and central to the goal of advancing value-based payment models. **While we believe the exceptions are largely appropriate as written, we urge ASTP/ONC to simplify, clarify, and align definitions and requirements across regulations.** Providers report ongoing confusion about how to apply and document exceptions, what constitutes an "unreasonable practice," and how "actual knowledge" is interpreted during enforcement. Concise, actionable educational materials and implementation guides – especially those designed for small and independent practices – are critically needed. We encourage the ASTP/ONC to directly engage practicing physicians in developing these resources to ensure relevance and usability.

The Task Force supports the development of a fair and balanced disincentives framework that promotes compliance with information blocking regulations for all parties, including payers and providers, while avoiding undue harm to small and independent practices.[10] **CMS should not remove physicians from VBC arrangements as a disincentive.** Doing so contradicts the agency's own goal of ensuring all clinicians participate in accountable care relationships by 2030. The Task Force believes that an effective disincentive policy should be scalable. **In lieu of sanctions for a first offence, providers who have committed clear information blocking activities should be required to submit a Corrective Action Plan (CAP) process that allows providers to remediate information blocking issues before penalties are imposed. We urge CMS and ASTP/ONC to monitor the impact of recently finalized disincentives – particularly on hospitals – and assess whether they effectively deter noncompliance.** Disincentives should be proportionate, account for intent, and not reinforce existing disparities in the health care system.

To build confidence in the information blocking complaint process, CMS and ASTP/ONC must ensure clarity and transparency in how complaints are handled.[11] One important step is to

---

[9] PR-12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

[10] PR-13. For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

[11] PR–14. How can CMS encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

prioritize enforcement actions involving "intentional" or "actual knowledge" of blocking, which would improve the ability to distinguish systemic issues from individual missteps. **Additionally, CMS should clarify how it will avoid issuing duplicative penalties for the same instance of noncompliance.** This transparency will help build provider trust in the system and encourage greater participation in the reporting process. Educational outreach and communication should emphasize how reported data will be used to improve compliance and identify areas for targeted support and education.

VIII. Digital Identity Credentials

Providers need assistance integrating third-party identity verification systems into existing EHRs and patient portals. CMS can drive this by updating interoperability requirements under the Health IT Certification Program to include support for federated identity protocols such as OpenID Connect and OAuth2. Additionally, CMS should provide implementation guides, pilot programs, and case studies such as the Department of Veterans Affairs' successful use of ID.me to help providers navigate the transition. By adopting a single, secure digital identity credential, CMS could reduce fragmentation and improve patient access to digital health tools.[12]

Having a single login process that would allow automatic login during a specified period would significantly improve physicians' workflows. The value of a common identity credential will grow as adoption increases across stakeholders. For widespread use, the credential must integrate seamlessly into existing provider technologies and not introduce new workflow burdens. **CMS should provide clear education for patients on how to obtain and use the credential, while also ensuring that the organization's offering digital identities are validated by independent third parties and held accountable for security and privacy.**

Similar standards and systems should be developed for providers and other health care workers to ensure a consistent identity framework across the health care system. **The Task Force supports the adoption of a federated, standards-based identity framework that enables payers and providers to jointly authenticate individuals and participate in trust ecosystems, without being displaced by intermediaries.** This approach supports secure access while avoiding unnecessary layers of manual attestations. It also avoids centralized credentialing models that introduce additional privacy and security risks.

Many providers will depend on their health IT vendors to implement digital identity capabilities.[13] If vendors are not adequately incentivized—or if incentives are structured poorly—they may implement the bare minimum required, leading to subpar user experiences and low adoption. Engaging end users can help ensure any digital identity credential system developed will center patients and physicians, while also prioritizing integration with existing clinical workflows to

---

[12] PR–9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

[13] PR–10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs): What are the challenges and benefits for providers?

mitigate additional administrative burden. **Therefore, CMS should consider policy levers to encourage vendors to prioritize user-centered design in implementing digital identity features. In addition, CMS should create legal safe harbors for providers who use CMS-approved identity services to reduce organizational hesitancy around adoption and liability concerns.**

The development of a nationwide provider directory with a centralized and credentialed identity framework could streamline data accuracy and provider information management. **However, CMS should avoid mandating participation until the system has been adequately tested and refined.** A phased approach, beginning with voluntary participation and robust feedback loops, will ensure the system adds value without increasing administrative burden. We recommend only mandating participation after a significant number of physicians have voluntarily entered their information. This will ensure the system functions well for its users and will drive design that provides value to physicians and other third parties.

Currently, provider information contained in existing systems is not reliable because it is not kept up to date consistently, contains inaccuracies, and is not consistent across different systems. Accurate provider information is integral for patients and consumers to access care. **CMS should first focus on ensuring data quality, clarity of roles and responsibilities, aligning regulatory requirements across programs (Medicare, Medicaid and Qualified Health Plan) and accountable governance before implementing new infrastructure or interoperability requirements.**

The Task Force supports the development of a standards-based, open access directory that includes validated FHIR endpoints, identity credentials and trust anchors, and TEFCA participation status and network affiliation. These features would help resolve long-standing challenges to endpoint discovery and credential validation, while supporting efforts to replace fragmented infrastructure with scalable, standards-based platforms. **The end goals should be for a national provider directory function as a federated infrastructure that can aggregate and index authoritative data sources without replacing or duplicating public facing health plan provider directories.**

The Task Force does not recommend combining FHIR addresses and digital identity credentials. The data stored by the digital identity provider should be only the minimum data necessary to provide the service. Additionally, data like FHIR addresses, physical addresses, and contact information will likely change many times over a physician's life, whereas the digital identity should not change, unless one must change identity providers. For these reasons, the Task Force supports separation between FHIR addresses and digital identity credentials. With that said, we suggest the agencies consider if the digital identity could be used to assert the authenticity of the FHIR addresses, given that those addresses could then be maintained in a national provider directory.

PAYERS

CMS seeks input on questions related to use cases and workflows that involve payers. The Task Force offers the following recommendations to ensure technical and policy alignment across federal programs, address barriers to participation, and support scalable, secure data exchange infrastructure across the payer ecosystem.

IX. Trusted Exchange Framework and Common Agreement

The Trusted Exchange Framework and Common Agreement (TEFCA) holds a unique role in promoting interoperability by creating a national framework and standards for health data sharing and connecting health information networks (HINs) across the country. Interoperability is key to an efficient and accurate patient- centric digital health ecosystem. **The Task Force recommends that CMS build upon and expand the role and uses of TEFCA by (1) expanding TEFCA's permitted purposes to include health care payment and operations to incentivize adoption by payers, (2) further expanding supported use cases to include research, public health, and vital statistics, and (3) leveraging TEFCA to support data sharing between and among federal agencies and health care entities to serve multiple purposes.**

For TEFCA to realize its vision of secure, trusted, and widespread nationwide data exchange, several technical and policy barriers specific to payers must be addressed.[14] The current TEFCA definition of Health Care Operations (HCO), which only refers to a portion of the HIPAA definition, may inadvertently prevent payers from receiving necessary clinical data. While TEFCA permits payers to request data, it does not explicitly require providers to respond, limiting data exchange that is essential to care coordination and transparency. **The Task Force recommends that the Recognized Coordinating Entity (RCE) update the HCO definition and operating procedures to explicitly require providers to respond to valid, payer-initiated requests for data exchange. In addition, the Task Force is concerned that the latest Sequoia Project standard operating procedure would allow providers to charge payers for data, which would significantly limit payer uptake.**

The Task Force also calls for clarity around HIPAA-related responsibilities and data disclosure governance. CMS, ASTP/ONC, the Office for Civil Rights (OCR), and the RCE should update TEFCA governance documents to clearly define the responsibilities of Qualified Health Information Networks (QHINs) and participants regarding data disclosures and third-party vetting. This includes defining responsibility in scenarios such as security breaches at a participant endpoint or a QHIN mistakenly admitting a threat actor. CMS should also allow payers to connect to more than one QHIN. Ensuring clear liability guidelines and aligning with HIPAA protections will help build trust in the TEFCA framework.

Similarly, CMS should clarify guidelines about the revised "minimum necessary" data definitions that create reciprocal response obligations for all participants. Data sharing between providers

[14] PA-1: What policy or technical limitations exist in TEFCA for payers, and how should they be addressed?

and payers typically falls under healthcare operations, subjecting it to minimum necessary rules. Under current rules, providers can decline to respond to health plan operations requests, while plans are obligated under TEFCA to respond to provider requests. Providers often take a cautious approach when sharing data with payers due to these constraints. Many providers and EHR vendors express legitimate concerns about granting patient access to information via TEFCA without robust authentication tied to known healthcare provider credentials. The prevailing workaround (using TEFCA for record location while requiring patients to authenticate separately to each identified organization via SMART on FHIR) significantly diminishes TEFCA's intended value for patient access by creating friction that defeats the purpose of a unified framework. To promote healthier data exchange, CMS should explore ways to address providers' concerns about minimum necessary requirements, enabling more open and comprehensive data sharing.[15]

In addition, TEFCA must strengthen its security protocols through continued participant and endpoint validation and certification. The Task Force supports updating Uniform Data Authorization and Policy (UDAP) standards to ensure enforceable data-sharing requirements and hold providers accountable for responding to authorized data requests. On the patient access front, while TEFCA's Individual Access Services (IAS) standard enables identity proofing, it currently lacks clear provisions for managing data-sharing limitations based on patient preferences. Payers need the ability to apply such limits, which requires a standardized classification system and alignment with internal consent frameworks. **We recommend CMS harmonize TEFCA's IAS with Patient Access API identity credentialing protocols to ensure consistency and efficiency.**

Finally, education, incentives, and engagement are needed to drive voluntary participation in TEFCA. Many providers – particularly those not included in prior federal health IT programs – lack the resources to participate meaningfully. The Department of Health and Human Services (HHS) and the RCE should offer technical and financial support to providers and smaller payers to facilitate onboarding. Additionally, QHIN membership fees must be reasonable and non-duplicative. A cost-effective approach is essential to foster broad adoption across payers, providers, and data partners.

X. API Implementation

While payers have shown support for CMS-mandated APIs—particularly the Patient Access API—wider adoption and use remain limited due to a lack of patient engagement and provider integration.[16] Most notably, the Patient Access API has seen low patient uptake, as many individuals continue to rely on or prefer provider-recommended portals to access their health information. **The Task Force recommends that CMS support complementary, provider-mediated access models that align with patient behavior while maintaining FHIR compliance.**

---

[15] PA-7: How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal?

[16] PA-2: How can CMS encourage payer API implementation for patients, providers, and payers?

Enabling hybrid access pathways may improve engagement without undermining interoperability goals.

Second, for electronic prior authorization (ePA) APIs to deliver on their promise, provider adoption must be addressed. Although payers are required to implement and maintain ePA APIs, current policy does not require certified EHR vendors to enable connectivity, presenting challenges for payers that seek to reduce the provider burden associated with ePA. Furthermore, under current MIPS rules, providers only need to complete one ePA transaction per year—a threshold too low to drive meaningful change. **The Task Force urges CMS to implement stronger provider-facing incentives and clear interoperability requirements to ensure that providers can consistently use and benefit from the ePA API infrastructure.**

To achieve the full value of the Payer-to-Payer API, all health plans – including the Veterans Health Administration and the Department of Defense – must participate. Without universal participation, the goal of creating longitudinal, patient-centered records across coverage transitions will remain out of reach**. The Task Force also emphasizes the importance of CMS fostering alignment across federal payers to build a unified API environment.**

Finally, to ensure that APIs operate efficiently at scale, **CMS and ONC should continue to support the development of a national FHIR endpoint directory, using the FAST National Healthcare Directory Implementation Guide.**[17] A centralized validated directory would significantly reduce the technical burden on payers and providers while supporting broader goals of interoperability, real-time data access, and care coordination.

XI. Digital Identity Credentials

As discussed previously in this comment letter, a trusted digital identity framework can streamline access for patients while strengthening data security. **The Task Force encourages CMS to promote adoption of digital identity credentials among payers and providers.**[18] CMS should support payer education and engagement efforts to build awareness and trust in these technologies. Increased familiarity and comfort with digital identity credentials will support more seamless API access across the health care ecosystem.

TECH VENDORS, DATA PROVIDERS, AND NETWORKS

CMS seeks input from all stakeholders on questions related to use cases and workflows that involve tech vendors, data providers, and networks. In our comments, we highlight specific opportunities where CMS and ONC can strengthen data access, promote standards alignment, and accelerate the adoption of key technologies across the health ecosystem. Our

[17] PA-4: What value would a nationwide provider directory with FHIR endpoints and digital credentials provide?
[18] PA-3: How can CMS encourage payers to accept digital identity credentials from patients and their partners?

recommendations emphasize the need for timely and coordinated implementation of federal policy, transparency in data exchange infrastructure, and support for tools that drive real-world provider adoption.

XII. Admit, Discharge and Transfer (ADT) Notifications

Access to timely and actionable health data is essential to enable developers to build tools that support coordinated, high-quality care.[19] Among the most valuable data sources are ADT notifications, which help developers build systems that alert providers and payers to patient transitions in care. Currently, CMS regulations require ADT data exchange only between hospitals and community providers. This limitation has led health plans to engage in fragmented, one-off contracts with individual health systems to access these notifications, impeding scalability and efficiency.

To better support care management and VBC, the Task Force recommends that CMS allow ADT data to be available broadly to those accountable for the care and/or outcomes of the patient – including the patient's primary care physicians and other clinical care teams, as well as health plans. Doing so would empower developers to design tools that deliver timely notifications to providers and care teams, improving care coordination and helping reduce avoidable hospital readmissions. Expanding ADT data access is a key opportunity to enhance system-wide responsiveness while advancing the goals of population health.

XIII. FHIR Endpoints

A centralized, trusted national directory of FHIR endpoints would significantly enhance health data exchange by providing developers and health care organizations with an authoritative source for discovering and verifying API connection points.[20] Today, the absence of such a directory leads to duplicated efforts, data exchange delays, and increased operational burden on developers and implementers.

The Task Force supports CMS and ASTP/ONC efforts to advance a scalable and secure FHIR endpoint directory as part of a modern interoperability infrastructure. A nationwide directory would streamline connections between systems, reduce the administrative complexity of point-to-point integrations, and facilitate more consistent, reliable API-based data sharing. This foundational tool would accelerate the adoption of FHIR-based technologies and strengthen the ecosystem's ability to support real-time, patient-centered data exchange.

---

[19] TD-2: What CMS and other data sources are most valuable to developers, and what obstacles limit access?

[20] TD-5: How could a nationwide directory of FHIR endpoints improve health data access?

XIV. Bulk FHIR

The increased use of Bulk FHIR would enable interoperable population health use cases at scale, addressing current limitations in population health interoperability.[21] Non-BBulk FHIR is inherently patient-specific and cannot efficiently handle dynamic cohorts needed for population health management. Current Bulk FHIR implementations also lack the ability for consumers to fetch only "updated" data, requiring full dataset refreshes that create unnecessary data transfer overhead and limit real-time population health monitoring capabilities. Properly implemented Bulk FHIR would support the rapid iteration and measurement cycles essential for modern healthcare innovation, enabling weekly or monthly population health assessments rather than annual reporting cycles. This aligns with the need to move beyond traditional constraints that have limited healthcare technology innovation.

The primary disadvantage stems from architectural decisions made by EHR vendors. Most EHRs operate with separate application and reporting data layers, having built their patient access FHIR APIs on top of the application layer due to low data volumes. When implementing Bulk FHIR, vendors face two options: building on their existing FHIR infrastructure or rebuilding to utilize their reporting data layer. Most EHRs have chosen the first approach because it's cheaper to build, but this creates significant operational risks when running Bulk FHIR against production resources. Consequently, many EHRs have artificially limited Bulk FHIR to very small cohorts—often around 1,000 patients—which defeats the purpose of Bulk access for population health use cases. Instead, EHRs should build Bulk FHIR capabilities against their reporting data layers to handle larger patient volumes safely and efficiently. This architectural approach would separate Bulk operations from production clinical workflows while enabling the scale necessary for meaningful population health analysis.

While building Bulk FHIR capabilities against reporting layers is essential, EHR vendors have legitimate concerns about allowing unfettered external access even to reporting systems. A governance framework establishing enforceable operational requirements and limitations will be necessary to balance access with system stability and security. Although TEFCA could theoretically provide this governance, the Bulk FHIR use case doesn't align well with TEFCA's QHIN-oriented model. A lighter-weight, focused governance forum specifically designed for Bulk FHIR operations would likely be more effective and should be convened separately to address the unique technical and operational requirements of population-scale data access.

XV.Interoperability

USCDI has improved interoperability by establishing a standardized set of data elements to be shared across health IT systems.[22] These standards have brought consistency to what data are exchanged and have reduced ambiguity for implementers and developers. However, the impact

[21] TD-15: Regarding Bulk FHIR APIs: How would increased use of Bulk FHIR improve use cases and data flow? What are the potential disadvantages of their use?
[22] TD-7: How has USCDI improved interoperability, and what are its limitations?

of USCDI is hindered when updates to its versions are not adopted in a coordinated manner across federal programs.

The Task Force recommends that CMS and ASTP/ONC work together to ensure consistent and timely adoption of new USCDI versions across all applicable regulations and programs. Misaligned timelines create confusion, increase administrative burden, and delay innovation. Coordinated policy implementation would support smoother integration of evolving standards and reduce administrative burdens for vendors, health systems, and payers alike.

XVI. Electronic Prior Authorization

To fully realize the promise of the 21st Century Cures Act, ASTP/ONC must complete and implement certification requirements for electronic prior authorization (ePA) APIs, as discussed previously in this comment letter.[23] Under CMS rules, payers are required to make these APIs available, but provider adoption remains limited in part due to a lack of certified capabilities within EHR systems.

The Task Force recommends that ASTP/ONC finalize certification criteria requiring health IT developers to incorporate ePA API functionality within certified EHR systems. This alignment is critical to enabling providers to effectively use the APIs made available by payers. In addition, ASTP/ONC should explore incentives that encourage provider adoption and use of these APIs, helping reduce administrative burden, accelerate prior authorization response times, and improve overall clinical workflow efficiency.

XVII. Data Infrastructure

Current rules generally encourage scalable network participation, but the framework for understanding network models should be more nuanced than a simple point-to-point versus shared infrastructure dichotomy.[24] There are four distinct approaches to health information exchange:

- Pure Point-to-Point: Direct connections between two systems that are technically and contractually independent of any other connections.
- Governance-Only Network: Point-to-point connections that are technically independent but share contractual governance with similar connections (example: Carequality).
- Network with Shared Infrastructure: Connections route through a central hub while still being sent and received by individual participant systems, with shared contractual governance supporting network components (example: CommonWell).

[23] TD-10: What further steps should ONC take to implement API certification provisions of the 21st Century Cures Act?
[24] TD-16: What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

- Full Data Replication: Participating systems feed data to centralized network structures that provide access to other participants through query-based or proactive routing (example: many state/regional HIEs).

The primary tradeoff of shared network models is the lack of real-time, synchronous data exchange capabilities. Current networks require lengthy data reconciliation processes that limit their effectiveness for time-sensitive clinical workflows. This conflicts with the need for rapid iteration and real-time measurement capabilities essential for modern healthcare innovation.

Point-to-point models would be more workable with enhanced ASTP/ONC requirements governing how EHRs must accept data via APIs. Current limitations include insufficient API support for essential functions like clinical note creation, or excessive fees when API support does exist. These barriers prevent the bidirectional data exchange necessary for comprehensive care coordination.

Each approach may be optimal in different circumstances. The middle approaches—governance-only networks and networks with shared infrastructure—offer important alternatives to the extremes of millions of individual point-to-point connections or full data replication. Where stakeholder needs and system capabilities are met through governance-only or shared infrastructure approaches, costs can be significantly reduced relative to full data replication models while maintaining necessary flexibility and real-time capabilities. The key is to ensure that regulatory frameworks support multiple models rather than forcing artificial constraints that limit innovation and efficiency in health information exchange.

XVIII. Role of Policymakers

Setting aside legislative requirements that led to TEFCA's creation, CMS and ASTP/ONC's role should not be to promote or support specific data sharing networks as policy objectives in their own right.[25] Rather, where policy objectives require information sharing among healthcare stakeholders, agencies should determine which of the four network approaches described above (pure point-to-point, governance-only networks, networks with shared infrastructure, or full data replication) best fits the specific policy circumstances and goals.

It is appropriate for CMS to prioritize ecosystem solutions that effectively meet its policy objectives, provided this approach doesn't completely exclude other market-chosen solutions that satisfy regulatory requirements. This balanced approach fosters innovation while ensuring policy goals are achieved through the most effective means available.

CMS has established payer API requirements for prior authorization, with policy goals including greater transparency and reduced administrative costs. However, these APIs currently require point-to-point connections, which may limit actual adoption by consumers and providers despite

[25] TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

payers meeting their regulatory obligations. To facilitate adoption and advance the original policy objectives, CMS may find that network approaches become necessary.

In this specific example, a full data replication model would be unnecessarily complex and costly, while a governance framework—possibly enhanced with network-level service infrastructure—would likely be sufficient to achieve transparency and cost reduction goals. CMS could appropriately incentivize participation in such a network solely to promote the underlying policy objective, rather than to promote network participation as an end in itself.

This approach allows CMS to leverage ecosystem efficiencies where they serve policy goals while maintaining regulatory flexibility. Organizations should remain free to choose alternative approaches that meet the same regulatory requirements, ensuring that market innovation isn't stifled by overly prescriptive network mandates. The key is aligning network selection with specific policy outcomes rather than promoting particular network models for their own sake.

By focusing on policy outcomes rather than network promotion, CMS can achieve operational efficiency while supporting the diverse innovation ecosystem that drives healthcare technology advancement. This approach recognizes that different policy objectives may require different network approaches, and that regulatory frameworks should remain flexible enough to adapt to evolving technology capabilities and market solutions.

VALUE-BASED CARE ORGANIZATIONS

CMS seeks input from all stakeholders on questions related to use cases and workflows that involve VBC organizations. The Task Force appreciates CMS' commitment better integrating digital health management, care navigation, and emerging technologies into VBC models, and offers the following recommendations to guide policy and infrastructure improvements.

XIX. Supporting Innovation

Successful participation in VBC models requires a robust set of health IT capabilities. While these technologies are critical to success, **the Task Force emphasizes the importance of preserving flexibility for alternative payment model (APM) participants to select tools that best align with their workflows and patient populations.**[26] Mandating specific technologies can create unnecessary complexity and stifle innovation. Essential health IT capabilities include:[27]

- Aggregation and analysis of clinical and claims data
- Real-time alerts such as ADT notifications
- Feedback mechanisms for both providers and patients

[26] VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?
[27] VB-3. What are essential health IT capabilities for value-based care arrangements?

- Risk stratification tools to prioritize care management
- Tools for transitions of care and discharge planning
- Support for non-medical drivers of health assessments
- Bidirectional data exchange across providers and payers
- Case management tools
- Population health analytics

Task Force members are actively deploying innovative technologies to support VBC workflows, including, but not limited to:[28]

- **Dragon Ambient eXperience (DAX)**: AI-powered clinical documentation at the point of care
- **Innovaccer Gravity**: Unified data platform enabling AI-driven care coordination
- **Tom (Lumeris)**: Automated patient engagement through SMS for follow-up care
- **MamaLift Plus**: Digital therapeutic platform providing personalized mental health support
- **Curana Health solution**: Technology to detect potential diagnostic oversights in radiology
- **Digital prior authorization tools**: Streamline ePA submission and response workflows
- **Patient triage technologies**: Identify high-risk patients requiring clinical intervention

CMS can support continued innovation by supporting the use of this technology without overly prescriptive mandates about when and how technology is deployed. To effectively integrate emerging technologies into APM requirements, CMS must acknowledge that while EHRs remain necessary for documentation and compliance, they are insufficient for value-based care success. EHRs were designed for billing and documentation, not for proactive population health, real-time risk management, or integrated care coordination. APMs should incentivize platforms that aggregate data from multiple sources—EHRs, claims, labs, social determinants, and patient-reported outcomes—into unified longitudinal records that enable advanced analytics and AI-driven decision support.

APM frameworks must move beyond retrospective, process-based metrics to embrace real-time decision-making enabled by AI and analytics. This aligns with the need for rapid measurement cycles rather than annual reporting. APMs should reward AI-based tools that identify rising-risk patients, forecast utilization, and surface care gaps in real-time, enabling proactive rather than reactive care management.

APMs should encourage adoption of platforms that address EHR usability limitations through intuitive interfaces and interoperable APIs that facilitate care coordination without adding clinician burden. Additionally, digital patient engagement—particularly for chronic conditions and underserved communities—requires omnichannel outreach and remote monitoring tools that go beyond traditional patient portals.

[28] VB-3. What are essential health IT capabilities for value-based care arrangements?

XX. Financial Incentives

To accelerate adoption of digital health tools, CMS should implement targeted financial and operational incentives that reward measurable improvements in patient outcomes.[29] APM organizations should receive greater regulatory flexibility and reduced technology requirements compared to non-APM organizations, recognizing that their value-based arrangements already provide strong incentives for quality and efficiency. Rather than imposing additional burdens, CMS should streamline requirements to enable rapid innovation cycles essential for APM success.[30]

The Task Force recommends that CMS:

1. **Provide financial bonuses to Accountable Care Organizations (ACOs) that offer digital health management or care navigation tools to beneficiaries in collaboration with their provider network.**
2. **Encourage the use of patient-facing incentives to increase engagement with digital technologies.**
3. **Offer enhanced per member per month (PMPM) payments tied to patient use of these tools.**
4. **Strengthen technical and financial assistance for providers historically excluded from health IT adoption programs, following guidance from NAACOS, AHIP, and the AMA.**

In addition, CMS should support voluntary incentive programs such as shared savings bonuses for organizations that achieve quality improvements through digital innovation. **CMS should also promote cost-sharing arrangements, group purchasing models, and peer learning collaboratives that enable APMs to share best practices and implementation strategies.**

XXI. Data Access

To fully realize the benefits of these digital tools and insights, VBC organizations must have access to comprehensive, timely, and integrated data. Essential data types include claims data, EHR data from all relevant care settings, ADT feeds, referral feedback and closure, lab and imaging results, accurate and timely patient lists, performance feedback, cost and quality information on network referral options, and pharmacy data (including real-time prescription benefit information).[31] Organizations can further leverage AI and population health platforms to analyze large datasets, while HIEs HDUs can deliver actionable insights at the point of care.

---

[29] VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?
[30] VB-9. What technology requirements should be different for APM organizations when comparing to non-APM organizations (for example, quality reporting, and interoperability)?
[31] VB-4. What are the essential data types needed for successful participation in value-based care arrangements?

Well-coordinated HIEs are demonstrably transforming healthcare delivery by simplifying care coordination through better communication and real-time data sharing. For example, a patient's comprehensive medical history can be accessed during emergency visits or specialist consultations, leading to safer, more informed clinical decisions. With accurate visibility into a patient's medical history, providers can avoid ordering duplicate tests and procedures by accessing recent labs, imaging, and treatment notes.

Research consistently demonstrates HIE's tangible impact on patient outcomes and healthcare efficiency. Studies show that hospitals participating in an HIE significantly reduced 30-day readmission rates, compared with those that don't participate, with researchers attributing these results to better access to patient information across different healthcare organizations, enabling more informed decision-making that facilitates care coordination. Additionally, hospitals utilizing HIEs had more timely access to critical patient data, resulting in decreased intensive care unit (ICU) admissions.

Cost savings are equally compelling. HIEs are associated with a reduction of total costs of care, lab test costs, imaging test costs, and overall measures of return on investment. Systematic reviews confirm that low-quality evidence generally finds that HIE reduces duplicative laboratory and radiology testing, emergency department costs, hospital admissions, as well as improving public health reporting and ambulatory quality of care.

When aggregated with other sources, HIE data proves valuable for identifying health trends, managing chronic disease populations, and supporting public health surveillance. The potential benefits of HIE on healthcare have fostered its implementation and adoption in the United States, with particular strength in facilitating population health management initiatives.

Significant challenges remain in maximizing HIE's potential for patient access. While HIEs have successfully demonstrated value in business-to-business healthcare information exchange between providers, they have been primarily designed around provider-to-provider workflows rather than direct patient access. The same patient identity verification and authentication challenges that have limited TEFCA's patient access capabilities also affect state and local HIEs. While the infrastructure exists to support improved care coordination and clinical decision-making among healthcare organizations, broader patient-directed access capabilities require continued development to fully realize HIE's promise of truly patient-centered health information availability.

Technology enablers like digital identity credentials can also help improve the accuracy and efficiency of data exchange by reducing duplicate records and enhancing patient-matching across systems.[32] This supports better coordination and lowers the administrative load on ACOs.

[32] VB 14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

Several critical health IT functions remain unaddressed by current ONC health IT certification criteria.[33] These include large-scale data aggregation across systems, inter-organizational population health data exchange, and support for extracting large volumes of electronic health information. These functions are essential to population-level analytics and reporting for VBC programs and should be incorporated into future certification criteria.

The Task Force supports the use of payer- and provider-agnostic data intermediaries – such as HIEs or HDUs – to facilitate secure, person-centered data exchange across the ecosystem. These entities can provide governance and security while reducing administrative burden for ACOs and APMs.

XXII. Essential IT Capabilities

Success in value-based care requires a unified data platform that aggregates, normalizes, and integrates data from disparate sources—EHRs, claims, labs, pharmacy, and social determinants of health—into longitudinal patient records. This includes robust patient matching across communities of care through master patient indexes, overcoming the silos and fragmentation that limit traditional EHR systems.[34] This comprehensive data foundation enables deeper understanding of patient risk and care needs essential for VBC success.

Essential capabilities include predictive analytics, machine learning models, and risk scoring algorithms for accurately identifying high-risk, rising-risk, and care-gap populations. These tools must support dynamic, real-time insights rather than retrospective reporting, enabling proactive resource allocation and population management that aligns with rapid measurement cycles.

Centralized platforms for coordinating care teams, automating workflows, and managing interventions across all settings—inpatient, outpatient, home, and community—are critical. These systems must enable closed-loop communication, task assignments, referrals, and follow-up tracking to reduce fragmentation. Digital patient engagement tools including mobile apps, messaging platforms, remote monitoring integrations, and digital navigation capabilities help patients stay informed, adherent, and connected to care teams outside clinical settings.

Digital quality measurement (dQM) capabilities that generate real-time electronic clinical quality measures directly from data systems reduce manual abstraction and enable organizations to act on quality gaps efficiently. AI-enhanced clinical decision support embedded at the point of care can offer support with identifying care gaps, suggesting interventions, predicting deterioration, ultimately improving outcomes in scalable, sustainable ways.

Seamless data exchange requires robust FHIR APIs, HIE integration, and standardized data formats ensuring continuity of care and real-time collaboration. Performance management

---

[33] VB-6. What specific health information technology capabilities that could benefit APMs are not currently addressed by existing certification criteria and standards that should be included under the ONC Health IT Certification Program?

[34] VB-3. What are essential health IT capabilities for value-based care arrangements?

dashboards tracking clinical and financial metrics—quality, utilization, cost trends, shared savings performance, and equity—enable constant monitoring and course correction.

Systems must support HIPAA compliance, granular consent management, and secure communication across multiple organizations. While EHRs remain essential for documentation and compliance, VBC infrastructure must layer on top of and around the EHR—integrating with but not being limited by traditional EHR capabilities.

XXIII. Interoperability & Quality Measurement

Interoperability remains a challenge – especially the shift to electronic clinical quality measures (eCQMs).[35] To support a smoother transition to eCQMs, the Task Force recommends:

1. **Delaying new APP Plus eCQMs until at least 2027** to allow sufficient time for ACOs to prepare, especially given that 2025 contracts are already in place and infrastructure remains inconsistent.
2. **Allowing MIPS CQM reporting in 2025**, as many ACOs have invested in this approach and need continuity.
3. **Retaining Medicare CQM reporting as a permanent option** until eCQM implementation is feasible for all ACOs.
4. **Modifying Medicare CQM requirements to apply only to the ACO's assigned population**, rather than the broader assignable population, to reduce inaccuracies and burden.

Without these changes, some ACOs may drop small practices, undermining care coordination goals. CMS should provide sufficient time and support to ensure a smooth transition.

Finally, a national FHIR endpoint directory would be a critical infrastructure investment to advance interoperability.[36] By enabling more reliable connections among providers, payers, and care coordination tools, such a directory would streamline setup and reduce errors. To maximize its impact, the directory should include details such as provider participation in APMs, adoption of interoperability standards and data, and endpoint certification status. This transparency would strengthen the digital foundation of VBC and accelerate scalable data exchange.

Recognizing that VBC is not one-size-fits-all, CMS should tailor criteria and standards to align with the specific goals of each model.[37] To reduce administrative burden, any new requirements should include clear, actionable pathways for demonstrating compliance. A flexible, model-specific approach will help ensure that providers can focus on delivering high-quality care while effectively participating in VBC initiatives.

---

[35] VB-11. What specific interoperability challenges have you encountered in implementing value-based care programs?
[36] VB-15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?
[37] VB-13. What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

***

HCTTF appreciates the opportunity to provide feedback on the Health Technology Ecosystem Request for Information. Please contact the Interim Executive Director Theresa Dreyer (theresa.dreyer@hcttf.org) with questions related to these comments.

Katie Boyer, MPPA
Senior Director of Policy and Government Affairs
agilon health

Sean Cavanaugh, MPH
Chief Policy Officer
Aledade, Inc.

Karen Johnson, PhD
Vice President, Practice Advancement
American Academy of Family Physicians

Melanie Phelps, JD
Senior Advocacy Advisor,
Health System Transformation
American Heart Association

Kevin Joyce
Vice President, Insurance Networks and Business Advisory Services
Atlantic Health System

Ashley Yeats, MD
Vice President of Medical Operations
Blue Cross Blue Shield of Massachusetts

Todd Van Tol, MBA
Executive Vice President, Health Care Value
Blue Cross Blue Shield of Michigan

Paul Hendley
Management Consultant
Blue Cross Blue Shield of North Carolina

Zak Ramadan-Jradi, MD, MBA
Head of Network Management & Strategy
Cambia Health Solutions

Justin Weiss
Co-founder, Head of Corporate Strategy
Charlie Health

Todd Gottula
Founder, President
Clarify Health

Brandon Wilson, PhD, MHA
Senior Director of Health Innovation, Public Health, and Equity
Community Catalyst

Joshua Habib, MBA
Senior Vice President
Covera Health

Amy Kaszak
Executive Vice President of Strategic Initiatives
Curana Health

Keith Shah, PhD, MBA, FACHE
Chief Operating Officer, Caremark Health Plans & Federal Employee Program
CVS Health

Mark McClellan, MD, PhD
Director
Duke Margolis Center for Health Policy

Erin Smith, JD
Vice President, Payment & Innovation Strategy
Elevance Health

Ashley Ridlon, MS
Vice President, Health Policy
Evolent

Sophia Tripoli, MPH
Senior Director of Health Policy
Families USA

Kendall Pittinger
Managing Principal of Payer Partnerships
Headway

Chris Stanley, MD, MBA
President of Populance
Henry Ford Health

Richard Lipeles, MPH
Chief Operating Officer
Heritage Provider Network

Aaron DeBoer, MBA
Executive Vice President
Honest Medical Group

David Nace, MD
Chief Medical Officer
Innovaccer

Anthony Barrueta, JD
Senior Vice President, Government Relations
Kaiser Permanente

Ryan Anderson, MD
Vice President, Clinical Care Transformation
MedStar Health

Sinsi Hernández-Cancio, JD
Vice President for Health Justice
National Partnership for Women & Families

Alan Balch, PhD
CEO
National Patient Advocate Foundation

Michael Eaton
Senior Vice President of Strategy, Interim Boston Market President
Navvis

Joy Burkhard, MBA
Executive Director
Policy Center for Maternal Mental Health

Seth Edwards, MHA
Vice President, Population Health and Value-Based Care
Premier

Melanie Matthews, MS
CEO
PSW

Sam Holliday, MBA
CEO
Oshi Health

Anthony Lam, MD
Chief Medical Officer
OPN Healthcare

Anna James, MBA
Senior Vice President, Government & Community Relations
Sentara Health

Chrisopher Riopelle, JD, MBA
Co-founder, CEO
Strive Health

Jim Sinkoff, MBA, CPA
Deputy Chief Executive Officer and Chief Financial Officer
Sun River Health

Kristen Mucitelli-Heath
SVP, Population Health, Advocacy
St. Joseph's Health / St. Peter's Health Partners
Trinity Health

Jessica Gubing, MS
Chief Executive Officer
UAW Retiree Medical Benefits Trust

Judy Zerzan-Thul, MD, MPH
Chief Medical Officer
Washington State Heath Care Authorit